[redacted]

DOCTORS PROGRESS & RE-ASSESMENT NOTES

[redacted]

IP No : 26/110098 UHID : 101301186

Name : [redacted]

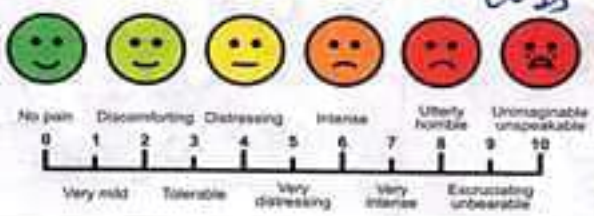
Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

Treating Dr/Dr.General Medicine Unit III \General Medicine

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	C/S/B JR on duty Medicine	
11/2/26 7:30pm	Patient Rina Kumari 38/F, normotensive non-diabetic presented to OPD with chief complaint of -	
	(2) (1) Black colored stools x 20 days	
	(1) (2) Pain Abdomen x 20 days	
	HOPI Patient was apparently well 20 days back when she developed -	
	(1) Black colored stools x 20 days (Brown)	
	- gradual onset, & non-progressive nature	
	- 3-4 episodes/day.	
	- dark Black tarry stools	
	- foul smelling	
	Ass w constipation.	
	- ass w generalised weakness.	
	- ass w decreased appetite	



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

[redacted]

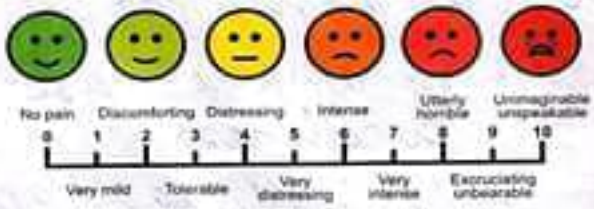
IP No : 26/110098 UHID : 101301186

Name : [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-S-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	(2) Pain in Abdomen x 20 days - ~~diffuse type~~ sudden onset, progressive in nature - diffuse type - mild-moderate intensity - aggravated on food intake - relieved on defecation - ass w fever (Non-documented) on/off NOC H/O ~~Bilvomiting~~ - ass w ~~Burning~~ micturition NO H/O vomiting  	(3) Burning micturition x 1 week a/w polyuria ↑ frequency No dysuria, hesitancy (?) not a/w fever

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DOCTORS PROGRESS & RE-ASSESMENT NOTES

IP No : 26/110098 UHID : 101301186

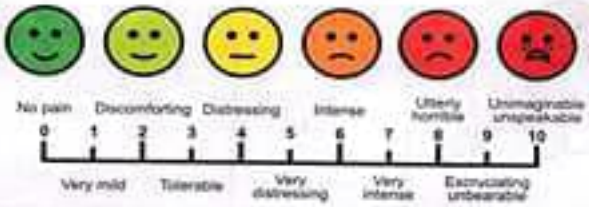
Name : [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-U1-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	Past H/O: No H/O T2DM/HTN/TB/asthma/KAD H/O Prev. Blood Transfusion (+) 1 year ago. 2 PRBC transfuse Surgical H/O: Open Appendectomy 18 yrs ago. H/O C-section (twice – Son (13 yr) daughter (14 yr.)) LMP: 2 Jan 2026. Family H/O: No H/O BT / Similar illness Personal H/O: Non-alcoholic, Non smoker. Veg by diet Bowel – altered Bladder – (N) Appetite – (N) Sleep – increased.	



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

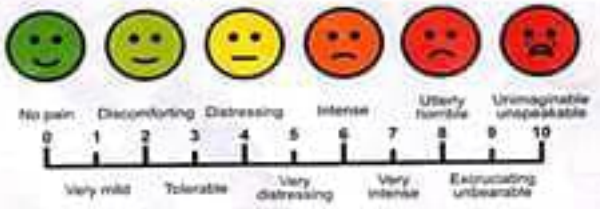
IP No : 26/110098 UHID: 101301186

Name : [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	GPE: P I Cy Cl L E (+) (−) (−) (−) (−) (−) pt. is conscious, oriented, to T/P/P. O/E: CNS: conscious, oriented CVS: S_1 S_2 heard R/S: B/L AE (+) PA: Soft, Non distended, BS heard, mild diffuse Tenderness (+)  	Vitals BP: 112/75 mmHg PR: 132 bpm SpO_2: 98% on RA Temp: 97.3°F RBS: 85 mg/dl

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DOCTORS PROGRESS & RE-ASSESMENT NOTES

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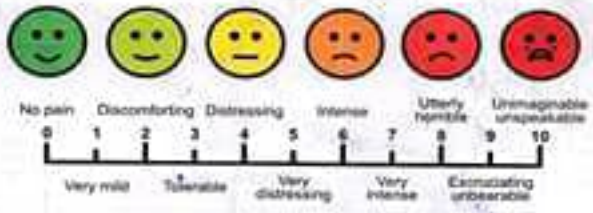
Name : [redacted]
Age/Sex :38 Y/F
Bed/Ward :FGN-W1-UT-5-65 \ General Medicine
DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	Prev. Reports USG-WA: 11/2/26: Liver raised periportal echogenicity ? Subjective ? viral etiology. - Impression: No significant Abnormality 5/11/23 Hb: 3.9, TLC: 2900, MCV: 130.5, Plt: 0.79 Blood group: O +ve 8/2/2026: Hb: 5.9, TLC: 4000, Plt: 1.37 Hct: 20.9, MCV: 99	



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

IP No : 26/110098 UHID : 101301186

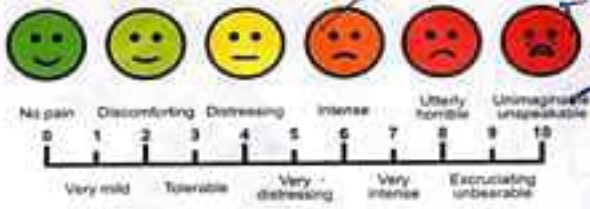
Name : [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	C/S/B JR on duty medicine	
11/2/26 8 PM	Δ S. Anemia ↓ evaluation	I/O charting RBS 3 O/day soft diet vitals monitoring
	Adv - CBC c̄ PBS - Iron Profile - S. ferritin - Retic count - Stool for OB - LFT - KFT - ECG - CXR-PA - Urine-Rm C/S - viral markers - ESR - CRP - B: LDH - Coombs test - direct - indirect - Blood group	Rx - INJ. PANTOP 40 mg IV OP Δ INJ. EMSET 4 mg IV SOS TDS. - Inj BUSCOPAN 1 Amp IV SOS. Arrange & transfuse 2 PRBC



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

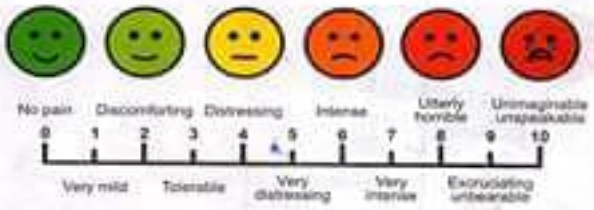
IP No : 26/110098 UHID : 101301186

Name : [redacted]

Age/Sex : 38 Y/F

Bed/Ward : FGM-W1-UT-5-65 \ General Medicine

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	C/S/B JR on duty	Medicine/AP Unit 3
12/2/26 8Am	Δ S. Anemia ↓ walution	I/O charting strictly. RBS 3 0/day. soft diet vital monitoring.
	O/E:	
	CNS: conscious, oriented	Vitals
	CVS: S_1, S_2 heard	BP: 108/60 mmHg
	R/S: B/L A/E (+)	PR: 92 bpm
	PA: Soft, BS (+), ND mild diffuse tenderness (+)	SpO_2: 98% on RA
		Temp: 97.5°F
		FBS: 87 mg/dl.
		I/O: 550/400



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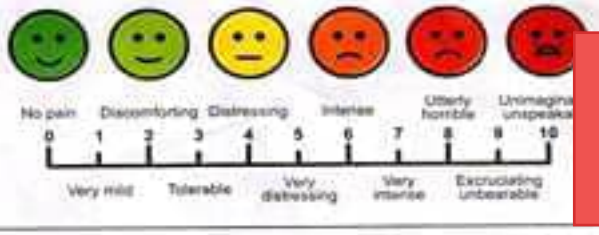
DOCTORS PROGRESS & RE-ASSESMENT NOTES

Patient Name: ..... Age & Sex .....

UHID No. : .....

Ward & Bed No. : ..... Unit .....

DOA ..... Time .....

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	Adv Arrange & transfuse 2 PRBC transfuse 1 PRBC IV infusion pump - [illegible] - [illegible] - S. Vit B12 - S. Folic acid - Stool for ova cyst  [redacted]	Rx - INJ. PANTOP 40 mg IV OD - INJ. EMSET 4mg IV TDS - INJ. BUSCOPAN 1 amp IV SOS

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DOCTORS PROGRESS & RE-ASSESMENT NOTES

[redacted]

IP No : 26/110098 UHID : 101301186

Name : [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

Doctor:Dr. Dr.General Medicine Unit III \General Medicine

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
12/2/26 1:00 pm.	[redacted] BLOOD CENTRE DONOR UNIT No. 065 License No. 724 -B(H) 'O' Rh D Positive/Negative Whole Blood/PRBC/FFP/PC (RDP) IP Volume 307 ml with SAGM/without SAGM CPD/CPDA Anti Coagulant 14% v/v Date of collection 16/01/26 Date of Expiry 27/2/26 Non Reactive For HIV 1 & 2, HBsAg, HCV, VDRL & Negative for Malaria Parasite Patient Name [redacted] UHID No. 101301186 W/o, S/o, D/o [redacted] Ward FMW Bed No. 12 Cross Match on 12/2/26 Cross Matched by [redacted] Compatible – Yes ✓ No ☐ Storage Temp. Whole Blood/PRBC/Thawed Plasma/FFP 2°C to 8°C, PC (RDP) 20°C to 22°C Voluntary Donation ✓ Replacement Donation ☐ • Use Disposable Sterile Transfusion Sets with filters only. • Blood/ Blood Components must not be used for Transfusion, if there is any visible evidence of deterioration like haemolysis, clotting or discoloration. • Do not add any medication. • Administer at room temperature. • Shake gently before use. • Do not dispense without prescription. • Properly identify the intended recipient Date of Issue 12/2/26 Time of issue 12:15 hrs Issued by [redacted] DCN/SGTMCHRI/F&F/VER.02/REV.01/381 If no reaction present, continue BT at 10-12 drops/min & finish BT in 3-4 hrs via infusion pump. Pre-BT vitals BP: 108/60 mmHg PR: 92 bpm SpO2: 98% on RA Temp: 98.7°F No pain, Discomforting, Distressing, Intense, Utterly horrible, Unimaginable unspeakable 0 1 2 3 4 5 6 7 8 9 10 Very mild, Tolerable, Very distressing, Very intense, Excruciating unbearable	DOC: 16/1/26 DOE: 27/2/26 DOT: 12/2/26 Blood Bag no: 065 Blood gp. O +ve Start BT initially at 3-4 drops/min observe for any reaction like fever rash, Tachycardia, If present reaction then stop BT immediately give Inj. AVIL 1 amp IV stat Inj. HYDRO (IRT) amp IV stat & Inform Dr. Post-BT vitals BP: 102/70 mmHg PR: 78 bpm SpO2: 98% on RA Temp 98.6°F

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DOCTORS PROGRESS & RE-ASSESMENT NOTES

[redacted]

IP No : 26/110098 UHID : 101301186

Name : [redacted]

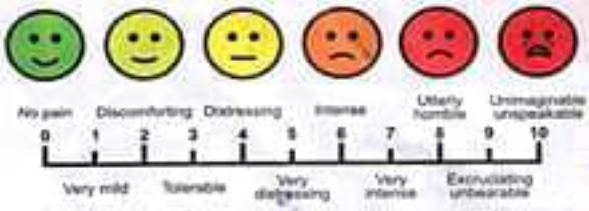
Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

Doctor :Dr. Dr.General Medicine Unit III \General Medicine

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
12/2/26 8pm	C/S/B JR on duty Medicine A/S. Anemia (Macrocytic) Evaluation (1 PRBC transfused) Vitals BP: 102/70 mmHg PR: 76 bpm SpO2: 97% on RA Temp: 98.6 F	Rx Continue same treatment Adv CBC c/m D1/7 Add → Inj ELDERVIT 2cc IV OD (AST) Add → Tab FOLIC ACID 5mg P/O OD



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

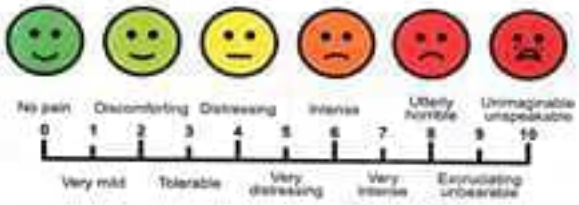
Patient Name: [redacted] Age & Sex .....

UHID No. : .....

Ward & Bed No. : ..... Unit .....

DOA ..... Time .....

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	C/S/B JR on duty / AP medicine	
13/2/26 8AM	Megaloblastic (Macrocytic) Anemia (1 PRBC transfused).	I/O charting strictly RBS 3 O/day soft diet vital monitoring
	O/E: CNS: conscious, oriented CVS: S_1 S_2 heard RS: B/L AE (+) PA: soft, BS (+), ND, NT Stool: not passed.	Vitals BP: 102/60 mmHg. PR: 98.2°F SpO_2: 98% on RA Temp: 98.2°F FBS: pt. refused. I/O: 1400/600



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DOCTORS PROGRESS RE-ASSESMENT NOTE

[redacted]

IP No. : 26/110098 UHID : 101301186

Name : [redacted]

Age/Sex : 30 Y/F

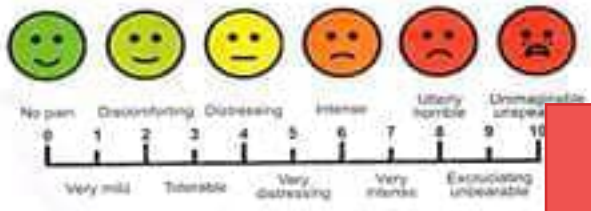
Bed/Ward : GM-V/1-UT-S-65 \ General Medicine

DOA/Category : 11/02/2026--17:24 / Ayushman

Doctor : Dr. Dr.General Medicine Unit III \General Medicine

& Sex.....

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
		Rx W/H [INJ. PANTOP 40mg IV OD] ↓ [INJ. ONSET 4mg IV ~~TDS~~ SOS] - INJ. BUSCOPAN 1amp IV SOS P2 - INJ. ELDERVIT 2 cc IV OD - TAB. FOLICACID 5mg P/O OD - Tab PANTOP 40 mg P/O OD BBF.
	Adv Psych opinion L/V/O depression → → ~~OBG~~	



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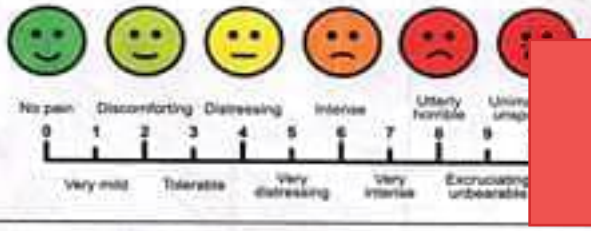
IP No : 26/110098 UHID : 101301186

[redacted]

Age/Sex :38 Y/F
Bed/Ward :/GH-W1-UT-5-65 \ General Medicine
DOA/Company :11/02/2026--17:24 / Ayushman
Doctor :Dr. Dr.General Medicine Unit III \General Medicine
Mobile No. : 6358637681

DOCTORS PROGRESS & RE-ASSESMENT NOTES

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	C/S/B JR on duty	medicine
13/2/26 8pm	S megaloblastic Anemia C/PRBC transfused)	Rx continue same treatment
	Vitals BP: 114/80 mmHg PR: 84 bpm SpO2: 98% on RA Temp: 98°F	



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

IP No : 26/110098 UHID : 101301186

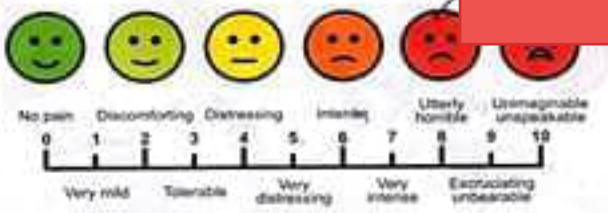
Name [redacted]

Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-5-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	JUSTIFICATION FOR WARD STAY Our patient Rina Kumari 38/F admitted in female medicine ward with Δ S. Anemia ↓ evaluation. Patient had Hb. of 5.6 on arrival. 1 PRBC was transfused on 12/02/2026. Patient was started on INJ. VIT-B12, folic acid and other supportive measures. Patient requires further stay in ward for treatment. Kindly approve for the same. Thanking you [redacted]	



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

IP No : 26/110098 UHID : 101301186

Name : [redacted]

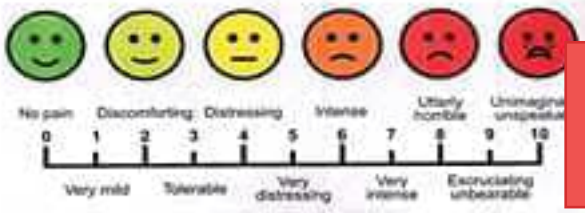
Age/Sex :38 Y/F

Bed/Ward :FGM-W1-UT-S-65 \ General Medicine

DOA/Company :11/02/2026--17:24 / Ayushman

Doctor :Dr. Dr.General Medicine Unit III \General Medicine

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
	JUSTIFICATION FOR LONG STAY Our patient Rina kumari 38/F admitted in Female medicine ward with Δ S. Anemia & evaluation. Patient had low hemoglobin of 5.6 on arrival. 1 PRBC transfused on 12/2/26 Patient was started on Inj. VIT B-12, folic Acid & other supportive measures. Patient requires further stay in ward for treatment. Kindly approve for same Thanking you	[redacted]



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

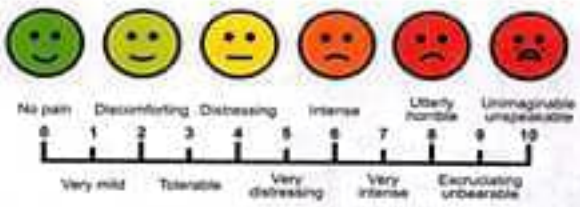
Patient Name: [redacted] Age & Sex

UHID No. :

Ward & Bed No. : Unit

DOA Time

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
14/2/26	C/S/B JR on duty / AP General medicine	
8Am	S Megaloblastic Anemia C/ PRBC transfusion	I/O charting vital monitoring Soft diet
	O/E: CNS: conscious, oriented CVS: S_1 S_2 heard RS: B/L AE(+) PA: soft, BS(+), NT, ND	Vitals BP: 128/70mm Hg PR: 76 bpm SpO_2: 98% on RA Temp: 98.2°F FBS: 88 mg/dl I/O: 1000/1150



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DOCTORS PROGRESS & RE-ASSESMENT NOTES

Patient Name: ..... Age & Sex .....

UHID No. : .....

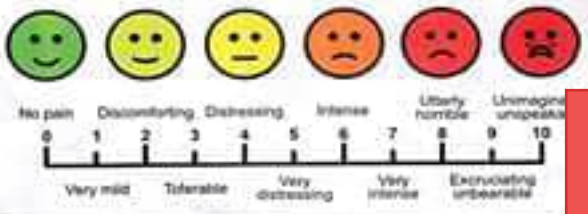
Ward & Bed No. : ..... Unit .....

DOA ..... Time .....

DATE & TIME	PROGRESS & RE-ASSESMENT NOTES	DOCTOR'S INSTRUCTION
		Rx - TAB. PANTOP 40 mg P/O OD BBF - TAB. FOLIC ACID 5 mg P/O OD Day 3 - INJ. ELDERVIT 2 cc IV OD - INJ. BUSCOPAN 1 amp IV SOS

Discharge
✓ Tab NEUROKIND PLUS P/O OD x 20 days
↗ tab FOLIC ACID 5mg P/O OD.

no sign of fever
wed & cxr
+ spare



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